Office Visit - Oct 23, 2026

with Joseph Sweet, PA at WashU Medicine Orthopedic Surgery

Notes from Care Team

Progress Notes by Joseph Sweet, PA at 10/23/2026 9:15 AM

NEW PATIENT VISIT

CHIEF COMPLAINT

Follow-up of the Left Foot

HISTORY OF PRESENT ILLNESS

History of Present Illness

Graham T Scanlon is a 27 year old male who presents for a second opinion for 6 months of intermittent left plantar second MTP joint pain.

Six months of intermittent pain localized to the plantar aspect of the left second MTP region. Initially felt like a possible metatarsal fracture with a marble sensation under the 2nd toe joint that improved. Ongoing pain with a snapping sensation near the joint with toe extension during gait; toe motion is largely tolerated when non-weightbearing. Walking triggers symptoms within 5 to 10 minutes with throbbing pain; he walks with a heavy limp and typically tolerates 15 to 20 minutes. He remains limited on weight bearing status through out the day on a daily basis due to symptoms. Pain varies day-to-day from 4/10 up to 10/10 and is sharp, throbbing, and nerve-like without burning, tingling, or hypersensitivity to touch.

Treatments to date include boot immobilization for over a month with partial improvement but rapid recurrence with toe flexion/extension with the boot, multiple radiographs reportedly without abnormality, and an MRI at Rayus reportedly read as normal with 2nd MTP swelling. NSAIDs have not helped. A fluoro guided steroid injection into the second MTP joint did not meaningfully improve walking pain but made symptoms calm down faster afterward. He has also tried physical therapy, shoe modifications including forefoot rockers, and an in-shoe pad or cushion for relief.

He also has intermittent flares of lateral foot pain at the site of a prior bone contusion that lasted a couple of months without x-ray damage, occasional mild discoloration with the left foot appearing darker in the mornings, and occasional similar but less severe pain at a comparable spot in the other foot.

No diabetes or tobacco use. No other significant episodic joint pains aside from mild intermittent hip discomfort.

Prior autoimmune workup including CBC, ESR, CRP, ANA, and rheumatoid factor without abnormalities per patient.

Denies smoking, diabetes, and autoimmune conditions.

PAST MEDICAL HISTORY
He No past medical history on file.

PAST SURGICAL HISTORY
He
Past Surgical History:

Procedure	Laterality	Date
• WISDOM TOOTH EXTRACTION *age 13 or 14*		

INITIAL REVIEW OF MEDICATIONS
He has a current medication list which includes the following prescription(s): azelastine, fluticasone propionate, mvn-min75-iron-iron ps-om3-dha, and sodium chloride.

DRUG ALLERGIES
He has no known allergies.

SOCIAL HISTORY
He
Social History

Tobacco Use
- Smoking status: Never
- Smokeless tobacco: None

Substance and Sexual Activity
- Drug use: Never
  Comment: CBD oil
- Sexual activity: None

Alcohol Use: Not on file

FAMILY HISTORY
His

Family History

Problem	Relation	Age of Onset
• Cancer	Sister	
• Anesthesia problems	Neg Hx	

REVIEW OF SYSTEMS

Please see Orthopedic Patient Questionnaire reviewed by me today.

PHYSICAL EXAMINATION

Physical Exam

CARDIOVASCULAR: Left dorsalis pedis pulse palpable.
MUSCULOSKELETAL: Point tenderness at the plantar aspect of the left second MTP joint; minimal additional tenderness at the left second toe PIP joint. No midfoot, sesamoid, first MTP, or great toe pain. Left heel cord tight.
- Pain with hyperextension and hyperflexion of the left second MTP joint, predominantly plantar, with no pain on grind testing. No pain with gentle range of motion of the left second toe except at end ranges, no pain with resistive flexion or extension, and no pain with lesser toe motion.
- Piano key test negative. Metatarsal squeeze negative.
NEUROLOGICAL: Sensation intact in the left foot.
SKIN: Minimal erythema at the dorsal PIP joints of the left second and third toes. No swelling or erythema of the left forefoot.

REVIEW OF X-RAYS/STUDIES

Results

Labs

CRP: Essentially zero/negative (per patient report)
ANA: Negative (per patient report)
Rheumatoid factor: Negative (per patient report)

Radiology

Left foot MRI: No fracture or bone marrow edema; periarticular swelling and increased signal at second MTP region; otherwise unremarkable. (Independently interpreted)
Left foot radiographs: No fracture; findings suggestive of disuse osteopenia; no second toe deviation. (Independently interpreted)

IMPRESSION/DIAGNOSIS/PLAN

I reviewed radiographs, physical exam findings, and treatment options with the patient today. We discussed the following assessment and treatment plan:

Assessment & Plan

Second MTP joint disorder of left foot

Chronic left plantar second MTP pain for 6 months, likely soft tissue or neuropathic etiology. Mild periarticular swelling on MRI. Conservative measures provided minimal relief.

- Requested transfer of outside MRI images from Reyes for review.
- Referred to Pain Management for evaluation and treatment indicated by exam.
- Initiated PRP consultation to discuss candidacy and costs prior to scheduling injection.
- Will discuss with the orthopaedic team regarding second MTP joint aspiration and possible coordination with Rheumatology; will follow up after clarifying process.

Left tight heel cord

Left heel cord contracture with reduced ankle dorsiflexion, likely contributing to increased forefoot loading and symptoms.

- Discussed considering physical therapy order for advanced gait analysis or offloading-focused therapy if a facility is identified and an order is requested. He will look into options and reach out if needed.

Patient voiced understanding and is agreeable to this plan. All their questions were answered at this time.

I was in collaboration with Dr. McCormick today.

Joseph Sweet PA-C, ATC, in collaborative practice with Dr. Jeremy McCormick
Physician Assistant, Foot and Ankle Service
Washington University Orthopedics.

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